LEARNER HANDOUT · YOUR CASE · NO MARKS

Malignant hypertherm

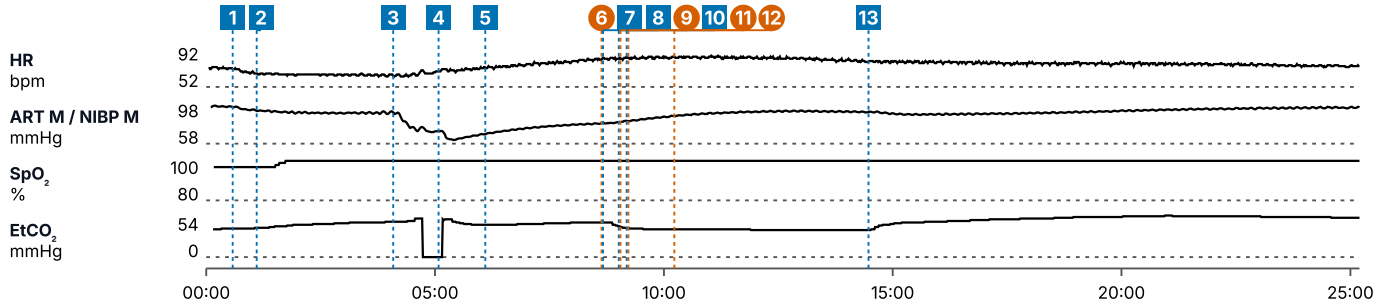
Fast-forwarded 5 min
19:45 → 24:45
No alarms

M 35 y 80 kg · 25:12 simulated

This sheet is yours to keep. It shows w ark you. The conversation, not this page, is the debrief.

Your run at a glance

■ team action · ● patient event · □ scenario state



Key moments

n	Time	Who	What happened
1	00:34	■ Team	Midazolam 2 mg and fentanyl 100 µg given, breathing room air
2	01:06	■ Team	Preoxygenating: 100 % O_2 by face mask
3	04:05	■ Team	Induced: propofol 2 mg/kg, succinylcholine 1.5 mg/kg (masseter rigidity); airway not yet secured
4	05:04	■ Team	Intubated: ventilator 12 × 500, PEEP 5, FiO_2 0.5; sevoflurane 2 % at 2 L/min
5	06:05	■ Team	Surgery under way: sevoflurane 2 %, rocuronium
6	08:38	● Patient	Early signs: $EtCO_2$ rising at the same ventilation, tachycardia
7	08:40	■ Team	Trigger off: sevoflurane stopped, O_2 100 % at 10 L/min, ventilation 20 × 600
8	09:00	■ Team	Dantrolene 2.5 mg/kg given
9	09:03	● Patient	MH treatment under way
10	09:11	■ Team	Cooling: 2 L saline at room temperature running
11	09:13	● Patient	MH treatment under way
12	10:13	● Patient	Recovering: $EtCO_2$ below 45

Questions to think about

1. What did you notice first on the monitor, and when?

2. Take one key moment: what were you thinking then, and what did you do?

3. One thing you will keep doing, and one thing you will do differently.

Take-home points

1. A rising $EtCO_2$ at the same ventilation with a rising heart rate: think of MH early.
2. Trigger off, 100 % oxygen at high flow, hyperventilation, and dantrolene 2.5 mg/kg.
3. Keep hyperventilating until the $EtCO_2$ has stayed down, and treat the potassium.

Next time, run the case again from its start and compare your key moments.